Knowledge, Attitude, and Practice Survey in Health Care – How Does it Matter?

Knowledge, attitude, and practice (KAP) surveys were initially started in the 1950s in the field of family planning and population research.[1] KAP surveys are now widely accepted method for the investigation of health-related behaviors and health-seeking practices. The aim of the KAP survey is to elicit what is known (knowledge), believed (attitude), and done (practiced) in the context of the topic of interest.[2] Information in the KAP survey is collected using semi-structured or structured questionnaires that are either self-administered or administered by interviewers.[2] Both qualitative and quantitative data are collected.

Knowledge

Knowledge is the capacity to acquire, retain, and use information. This requires a mixture of comprehension, experience, discernment, and skill. The nature of knowledge depends on the modes of acquisition of ideas: perception, imagination, memory, judgment, abstraction, and reasoning.[2] Knowledge criteria center around the sense that allows a person to distinguish between right and wrong, as studied by logic (deductive reasoning) and scientific method (formulating and testing hypotheses). Proper education is the prerequisite of knowledge. Basic intellect is essential to acquire knowledge.

Attitude

Attitude is a learned predisposition to respond in a favorable or unfavorable manner toward people, an object, an idea, or a situation.[2] A positive attitude is essential for a favorable outcome. Proper knowledge leads to a positive attitude.

Practice

Practice is the application of rules and knowledge that leads to action.

KAP survey provides quantitative and qualitative information. KAP surveys can reveal the misconceptions or misunderstandings that may represent obstacles to the activities that we would like to implement and potential barriers to behavior change. KAP survey essentially records the "opinion" of the participants and is based on their statements. Hence, there may be considerable gaps between what is said and what is done.[1]

Uses

KAP survey can be useful to measure the extent of a known situation/condition, confirm or disprove a hypothesis, and provide new horizons.[3] It may be useful to enhance the knowledge, attitude, and practices of specific themes to identify what is known and done about various health-related subjects. KAP survey can be useful to establish the baseline reference value for use in future assessments/research and help measure the effectiveness of health education activities' ability to change health-related behaviors. Analysis of the KAP survey can be useful to suggest an intervention strategy suitable for specific local circumstances and the cultural factors that influence them; and plan activities that are suited to the respective population involved.[3]

KAP surveys have become popular as they are reasonably easy to design, conduct, analyze, and interpret. The KAP survey provides valuable information for resource allocation, planning, and implementation of public health programs.[1] A KAP survey conducted before and after an awareness program or an intervention program can measure the effectiveness of the program. KAP survey can be useful to determine whether a program is performing in accordance with expectations and may be helpful to introduce midcourse corrections as appropriate.[1] Interim assessments can be useful to correct defects in these programs; the failure of which would otherwise be discovered only after program completion.

Important Indications for Conducting Knowledge, Attitude, and Practice Surveys in Health Care

The KAP survey is essential if no surveys have previously been conducted in the population of interest, if there are knowledge gaps despite previous surveys, or if there is a specific need.[4]

- To identify the baseline knowledge, myths, misconceptions, attitudes, beliefs, and behaviors with regard to a specific public health-care problem[1]
- To derive information on the needs, issues, and barriers to develop effective, locally relevant public health interventions and to develop strategies to overcome these barriers[2]
- To measure the effectiveness of intervention programs aimed at correcting and changing health-related knowledge, attitudes, behaviors, and practices and to identify any deficiencies so as to correct these in time.[2]

In this issue of the journal in an original article "A cross-sectional study on KAP about inhaler usage among

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asthmatic patients," authors have found gaps in these key areas, which require measures to improve the same.

Proper knowledge about the various aspects of a disease leads to a positive attitude and practice leading to better outcome. Patients with chronic diseases like asthma should be imparted with proper knowledge about the various aspects of the disease, including self-care, long-term management, and usage of medications. Misconceptions regarding medications for asthma and the usage of inhaled medications, in particular, are widely prevalent in Indian patients.[5,6] One of the main reasons for uncontrolled or partly controlled asthma is poor adherence to medications and improper usage of inhaled medications.[7] Hence, health education of asthma patients should focus on imparting knowledge on the proper use of asthma medications – inhaled medications in particular. Misconceptions and fear regarding the use of inhaled medication should be alleviated by proper counseling.

We conclude that the KAP survey is useful to derive valuable information about various aspects of health care and provide insights into interventions to improve the outcomes.

Financial support and sponsorship

Nil.

Conflicts of interest

There are no conflicts of interest.

M. Vishnu Sharma

Department of Respiratory Medicine, A.J. Institute of Medical Sciences and Research Centre, Mangalore, Karnataka, India

Address for correspondence: Dr. M. Vishnu Sharma, Department of Respiratory Medicine, A.J. Institute of Medical Sciences and Research Centre, Kuntikana, Mangalore, Karnataka, India.
E-mail: drvishnusharmag@gmail.com

Received: 29-10-2023
Revised: 02-11-2023
Accepted: 04-11-2023
Published: 04-12-2023

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Access this article online

Quick Response Code:

Website: www.jalh.org

DOI: 10.4103/jalh.jalh_52_23

How to cite this article: Sharma MV. Knowledge, attitude, and practice survey in health care – How does it matter? J Adv Lung Health 2024;4:2-3.